

Conditional Cash Transfer (CCT) Enrollment In The Philippines

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Abstract

The “war on drugs” started by the government to stop the drug distribution in 2016 among the country. Nowadays it is still an ongoing situation, meaning that the measures that the government has chosen might not have been effective, the killing of innocent people is still an ongoing issue occurring in the country, bringing poverty to it. The enrollment of people in the CCT is reflected by this situation.

-Dataset-

Philippines: Pantawid Indigenous People (IP) Data

(<https://data.humdata.org/dataset/philippines-pantawid-indigenous-people-ip-data>)

-Variables-

There are 16 variables in the dataset starting from Region Code, Region, Province Code, Province, City_Mun Code, City_Municipality, Barangay Code, Barangay, 0 to 2_Female, 3 to 5_Female, 6 to 18_Female, 19 & Above_Female, 0 to 2_Male, 3 to 5_Male, 6 to 18_Male, 19 & Above_Male,

The variables have the different entities to it;

- Region Code: This is the two-digit designated number allocated to the region itself, it ranges from 0 to 99.
- Region: It is the Philippines' standard geographic location divided by the regions.
- Province Code: This is the two-digit designated number allocated to the province itself, it ranges from 0 to 99. The province code is independent of the region code and if the province changes and shifts to the other regions its province code will remain the same.
- Province: It is the Philippines' standard geographic location divided by the provinces.
- City_Mun Code: This is a two-digit code that generally defines the relative alphabetical sequence of the municipalities within the province. It ranges from 01 to 99. Municipality Code 01 means it is the first municipality in alphabetical sequence within that province. The Municipality Code is dependent upon the Province Code to fully establish the identity of the municipality. This two-digit code is used to identify the municipalities, cities or municipal districts in a particular province. In the case of the first regional district (City of Manila) of the Metropolitan Manila Area (National Capital Region), the fourteen city districts of the City of Manila are treated as municipalities
- City_Municipality: It is the Philippines' standard geographic location divided by the city's municipalities.
- Barangay Code: This is the two-digit designated number allocated to the barangay within the city municipality, it ranges from 0 to 99.
- Barangay: Barangay is the lowest political administrative unit of the Philippine government.
- 0 to 2_Female: It refers to the number of female beneficiaries ages 0 to 2 years old
- 0 to 2_Male: It refers to the number of male beneficiaries ages 0 to 2 years old
- 3 to 5_Female: It refers to the number of female beneficiaries ages 3 to 5 years old
- 3 to 5_Male: It refers to the number of male beneficiaries ages 3 to 5 years old
- 6 to 18_Female: It refers to the number of female beneficiaries ages 6 to 18 years old
- 6 to 18_Male: It refers to the number of male beneficiaries ages 6 to 18 years old
- 19 & Above_Female: It refers to the number of female beneficiaries ages 19 and above.
- 19 & Above_Male: It refers to the number of male beneficiaries ages 19 and above.

-Audience-

CWC (Council for the Welfare of Children) - Usec. Angelo M. Tapale, executive director.

This organization takes control over decision-making and policy implementation for the children's welfare in the Philippines. The CWC works with other organizations, such as government agencies and civil society organizations, that will fight for children's rights and safety.

-Story Board-

- Issue: high enrollment on CCT in the Philippines.
- Demonstrate Issue: showing graph with the number of people registered in the CCT by region.
- Defined a few reasons why regions have high registration for CCT.
- Found articles and government publications that support our reasoning for enrollment of CCT.
- Compare the data given with the charts found in the articles and government publications, and show the correlation between them to demonstrate the issue.
- Recommendations: propose a few programs and policies to the CWC (Council for the Welfare of Children) in order to improve social and economic conditions that affect children, and introduce policies that focus on rehabilitation, for the government to not provide as much funding as they are currently.

-Introduction-

This presentation will talk about how the CCT is distributed all through the Philippines, and will go in depth of how that money is distributed between families that are considered 'poor' by Listahanan. There are two types of aids, one for education, and one for health. Those families that meet the qualifications for the aid, will be given P500 for education, and P300 for health. CCT stands for Conditional Cash Transfer Program, and is a program that has been incorporated in the Philippines to assist families that may have less resources. The dataset used focuses more on the MCCT, which stands for Modified Conditional Cash Transfer Program, and is a sub-component of the CCT where they target families that have other type of conditions, such as children from ages 0-18, homeless, indigenous peoples, and families in need of special protection (FNSP). IP stands for indigenous people and FDS stands for Family development sessions.

The data is about all the registered IP beneficiaries regardless of the program status. There are 17 regions and numerous provinces under them. Provinces have different cities/municipalities and inside of the cities/municipalities, there are different barangays.

In this presentation, we will be explaining the problems going on in the country that might cause the people to join the CCT program, and what could be done to lower the poverty, in order to lower the CCT rate, and hope for the safety and well-being of the children of the Philippines.

-Problem Statement-

What can be done to decrease the number of people enrolled in the CCT in the Philippines, by changing the way drug-stop intervention is done?

-Business Understanding-

The number of people enrolled in the CCT is distributed by regions of the Philippines. The CCT is provided by the government to help support poor people, homeless people, indigenous people

and children under the age of 18 and pregnant people. Our goal is lowering the enrollment of CCT to improve their lifestyle, but at the same time the government will be benefiting from this by funding less money towards it.

-3-minute story-

The main purpose of this research is to decrease the number of people that need to be enrolled in the CCT program by trying to improve the economic and social situation of the country of the Philippines. By doing so, the regions which have the highest number of people enrolled in the CCT will be analyzed. It shows that those regions are: autonomous region of muslim mindanao, Region XI (Davao region), and Region IX (Zamboanga peninsula). The reasons behind this high rate of enrollment are related to economic reasons, but mostly social issues. This is because in 2016 a war started, mostly affecting the regions in the south, but also the entire country. This war is called the “war on drugs”, and is a government attempt to stop the drug distribution and drug use from certain regions of the country. Even though the idea behind this execution is good, the way the government is taking measures is not appropriate. By killing innocent people and children for the only reason of being addicted to drugs, the people have started riots and protest against this measure, and some of the protests end up in the war, which is affecting thousands of people in the country. War brings poverty, and poverty leads people to enroll in the CCT. The government is the one offering the money to the CCT program, while the DSWD is allocating the resources, plus using money for this war. Therefore this presentation is trying to implement a new performance to improve people’s lives, and stop the spending of money by the government, maintaining the goal of stopping the drug distribution in the country by explaining the situations and the correlation between people registering in CCT and the “War on drugs” to the Council for the welfare of children. A suggestion of approaching these goals would be to stop murdering people for being drug users, and start the intervention of stopping drug abuse towards drug distributors. Drug users should feel safe enough to get help, rather than not say anything because they are scared the government is going to kill them. These new and better ways will help in convincing the government to make required changes in the ongoing process. By implementing these new concepts, people would gain trust again in the government, which would most likely lower the chances of war still happening, and therefore poverty rate would decrease, making people not needing to enroll in the CCT program, saving the government money.

-Big Idea-

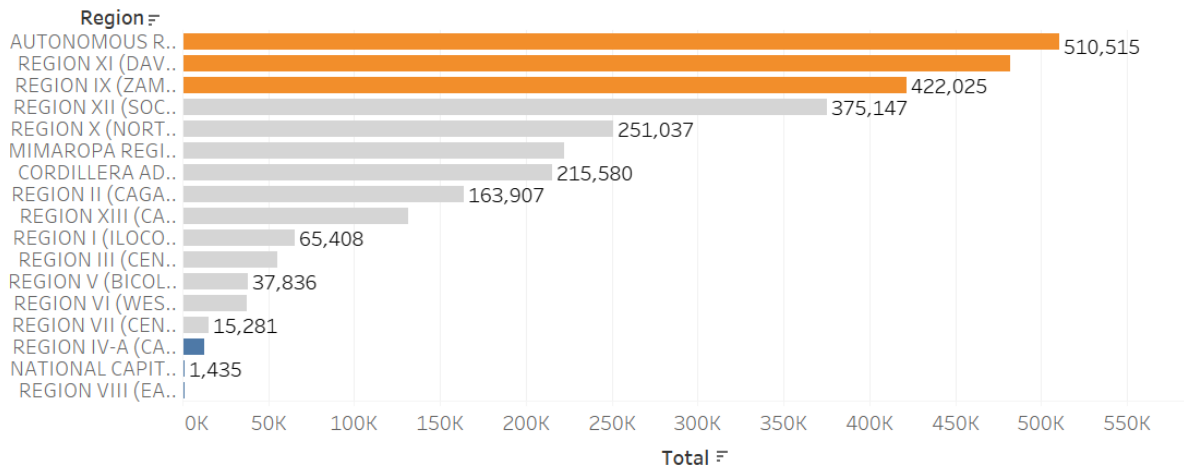
The issues for the highest number of people registered for CCT in particular regions like Autonomous region of muslim mindanao, Region XI (Davao region) and Region IX (Zamboanga peninsula) were identified as drug addiction among children, poor economy rates, high poverty rates and internal conflicts and riots with the government, these can be countered by putting a stop to executing the people (drug addicts) and instead helping them with rehabilitation by gaining their trust, and trying to decrease the poverty rates by implementing other ways of stopping the drug distribution and abuse.

-Data Preparation-

For the exploratory data analysis, we uploaded the dataset into python. Using code, we saw that there are no missing values for any of the variables. There are a total of 17312 rows and 17 columns. We will then perform visualization, targeting our business question.

-Data Visualization-

Distribution by region

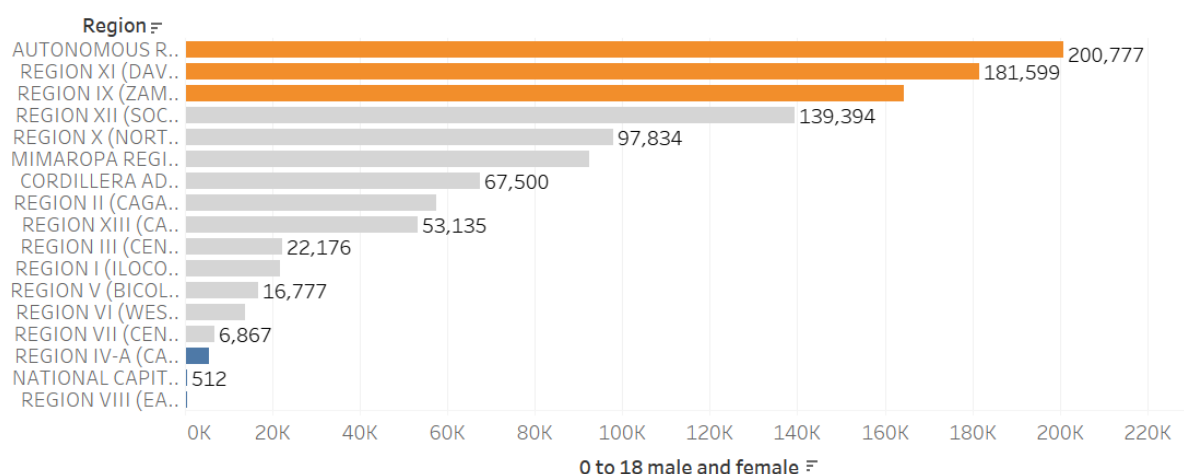


Graph 1: Distribution by region (0-19+)

The graph above represents the number of enrollments in each region for all ages.

The “Autonomous Region In Islamic Mindanao (ARMM)”, “Region XI (Davao Region)”, and “Region IX (Zamboanga Peninsula)” have the highest number of enrollments, with each region having more than 400,000 enrollments. However, the following regions: Region IV-A (Calabarzon), National Capital Region (NCR), and Region VIII (Eastern Visayas) show significantly lower enrollments.

Male and female between 0 to 18



Graph 2: Distribution by region (0-18)

The graph above shows the distribution by region of male and female individuals from 0 to 18. As it is shown, the same regions that were shown on the graph before are the regions with

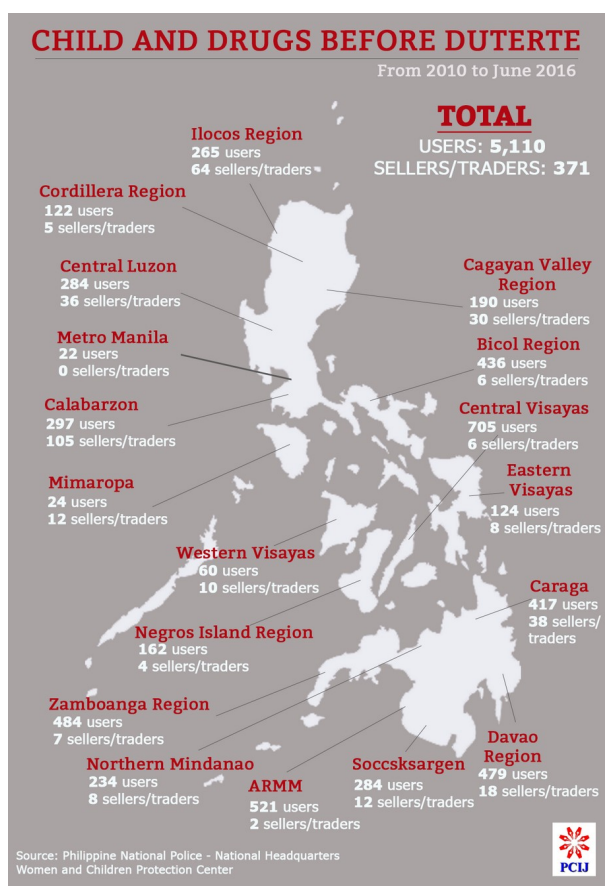
the top 3 and bottom 3 of people enrolled in the CCT. It is appreciated that the amount of people enrolled in the CCT compared with all the ages is lower, but is still a significant amount.

Regional enrollment



Graph 3: Regional enrollment

The graph 3 highlights the regions in the Philippines where the Koronadal region has the second largest number of the people registered with the amount rising to 1737. The Bacnotan region has the maximum number of people registered in it which is 27% more than the Koronadal region. The least amount of the people in the CCT region consists of the southern part of the region, part of the Philippines. The city part of the region shows us the spread of the population in the CCT across the Philippines.



Graph 4: Regional drug distribution and usage

Picture of drug use vs. drug trade(Lucero, 2016)

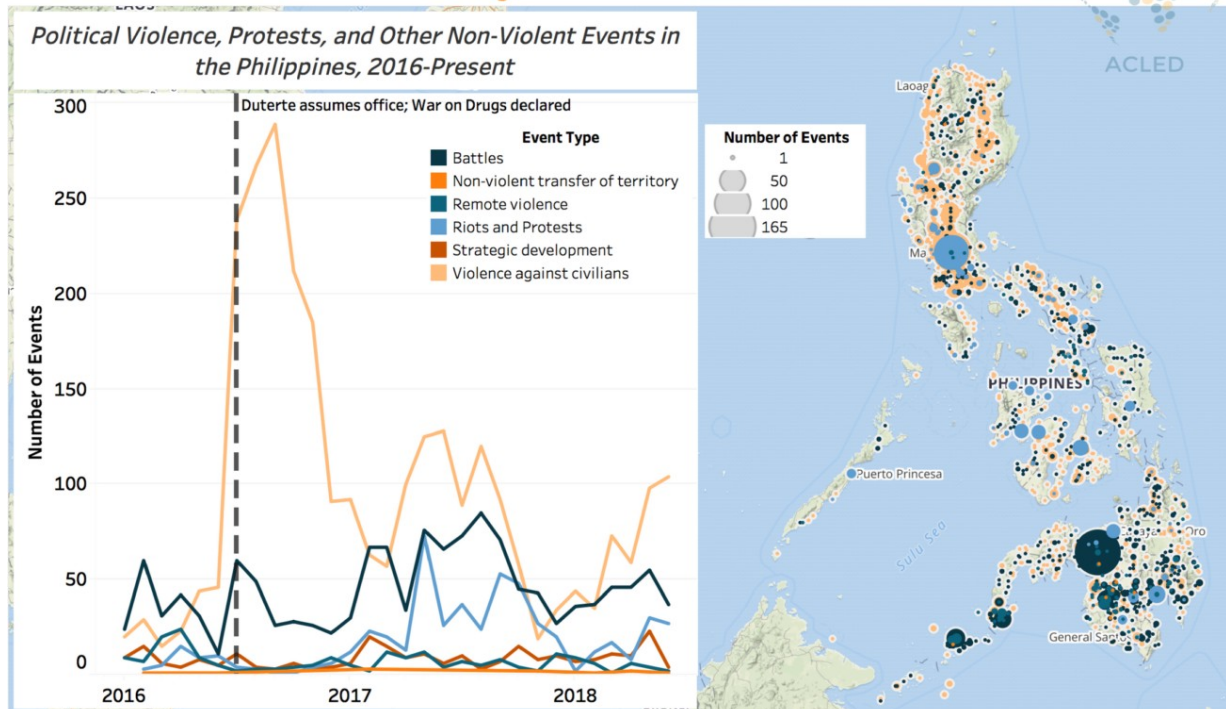
The visualization above shows the distribution of drugs and drug abuse all over the regions of the Philippines. Using this chart, the ratio can be calculated, in order to focus our research more towards our goal. As shown on it, there is a higher ratio of drug users over drug dealers in the south. ARMM has 260 drug users per 1 drug seller, meanwhile Easter Visayas has a ratio of 15.5 drug users over drug sellers. Overall, the ratio of the country is 13.77 drug users per drug seller. The territories in the north tend to have a lower ratio since the number of drug sellers, while in the south it is the opposite, less drug sellers and more drug users. These numbers match with the barcharts shown above, since drugs bring money, more people in the north are in no need of CCT, since there are a huge amount of drug sellers, meanwhile people in the south are more enrolled in the CCT since there is more poverty and drug usage.

Graph 5:

Graph showing political violence, protests, and other non-violent events in the Philippines, 2016-Present (Pavlik et al., 2020)

ACLED data for the Philippines now available for download

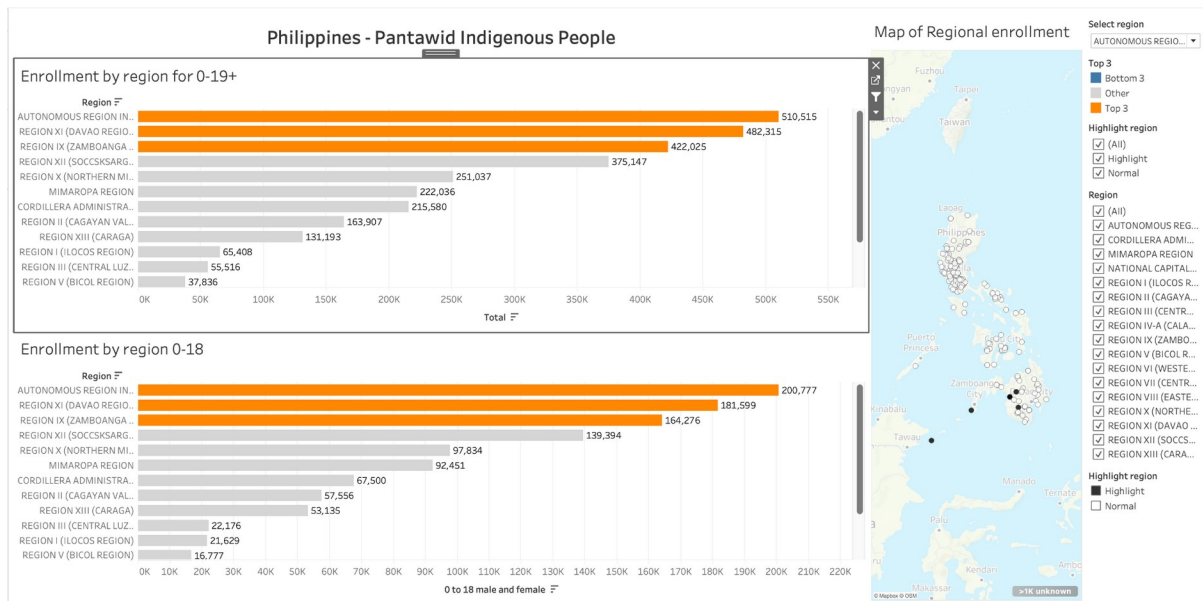
Coverage Available from 2016-Present



This visualization shows the different ways of how people are fighting against the “war on drugs”. Regions in the south show more battles, meanwhile regions in the north show more riots and protests. The entire country is suffering violence against civilians, but as it is shown, there are main cities where there has been the most conflict. Regions in the south, since there are more users than sellers, need to fight for their rights, and that is why battle is evolving there. Regions in the north have more sellers than users, therefore there is less violence, but the people are protesting against what is happening in the country, but mostly in the south.

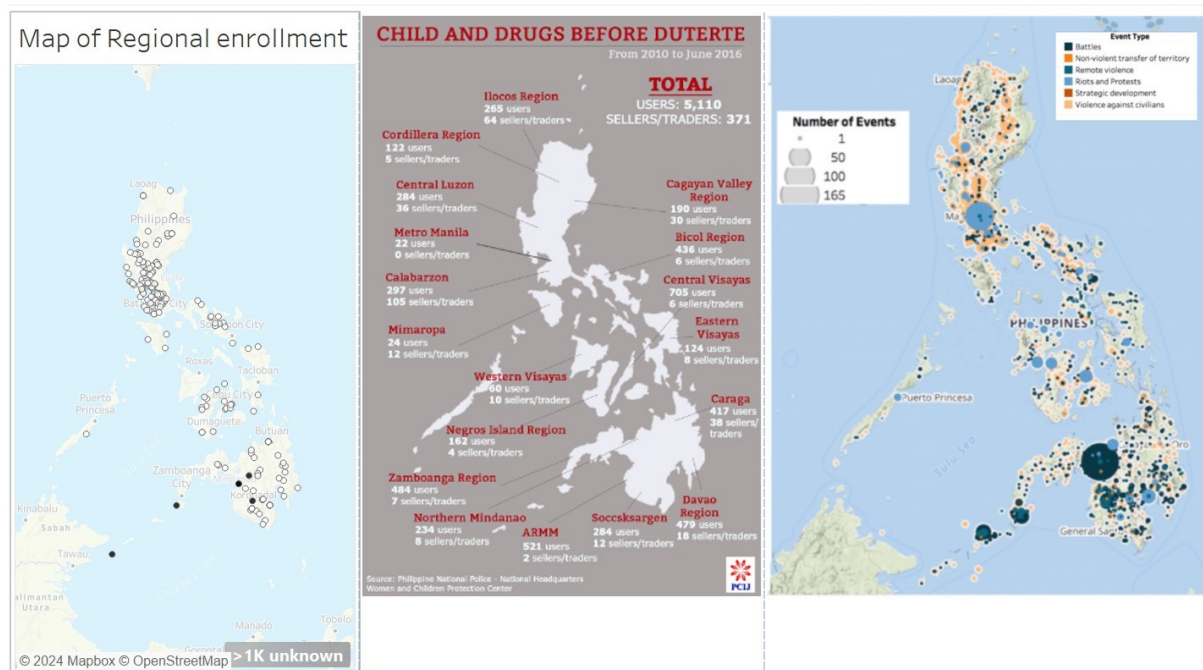
-Dashboards-

First Dashboard:



For this dashboard two horizontal barcharts and a map were chosen for the visualization. The horizontal barcharts show the number of people enrolled in the CCT, the upper graph shows all the ages (0-19+), and the bottom graph shows the children enrollment (0-18). The map shows the enrollment of CCT per region, and shows how it is distributed in the country. Preattentive attributes were used to differentiate the top and bottom 3 regions with the highest enrollment (orange for the top, and blue for the bottom, and gray for the regions that are not those ones). Gestalt principles are used in the barcharts, such as proximity, by adding the total enrollment by the end of the chart to make it easier for the audience to see and understand. A parameter was used in these graphs to create the coloring for the top and bottom regions, and a filter was made to show different regions as it is shown at the right of the dashboard.

Second Dashboard:



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For this dashboard three maps were chosen to express the information regarding our research goal. The map to the left was made by us, the middle map was obtained from “users, Pushers, runners” (Lucero, 2016), and the map to the right was obtained from *The Philippines - 2016 data release* (Pavlik et al., 2020). The gestalt principle of proximity was used in this dashboard by adding the three visualizations by each other, in order to show the correlation between them.

-Conclusion/Suggestions-

This presentation concludes by showing that the number of people registered in CCT directly correlates with the social issues happening in the Philippines since 2016. Primarily, the southern parts of the Philippines are observed to have a higher number of people registered for CCT, and one of the potential reasons for this could be the “War on Drugs” currently happening all over the Philippines but with intense protests and riots in the central part. The southern part has very intense battles taking place due to the measures the government has implemented to decrease the usage of drugs. The research revealed that the people suffering from the method

that the government is implementing are children below 18 years. It also supports this reasoning since the southern regions have a larger group of indigenous people residing in them. The drug use vs drug trade graph shows the regions which have a high user-to-seller ratio, which supports the data obtained from the Pantawid Indigenous People (IP) Data dataset. This graph supports our conclusion that the regions in the south, particularly the Autonomous Region of Muslim Mindanao (ARMM), Region XI (Davao region), and Region IX (Zamboanga peninsula) have a large number of people registering for CCT because of the higher user-to-seller ratios compared to the central and northern regions.

-Recommendations-

Through the findings of this research, a few recommendations have been suggested. Implementing policies focused on rehabilitation rather than executing drug users who are children below 18 will make for a better change. Trying to improve the economy rates in the regions with higher numbers of people registered for CCT will help the residents in being independent and will not require them to rely on the government for aid. Programs like helping with education, advising, job search and creating more job opportunities should be put forth to improve the economy rate. Healthcare is also a concern, therefore building more healthcare constructions, care centers and rehabilitation centers will help lower the mortality and drug usage rates. With programs like this, citizens will start trusting the government and believing that they can help them fight against drug addiction. This will shift the government's spending on people's welfare rather than on CCT aid. In turn, this will result in a lower number of people registering for CCT.

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